# Estimating Potential Unmet ADHD Assessment Demand in England

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## Executive Summary

ADHD services in England have experienced growing demand in recent years, alongside increasing public awareness, longer waiting lists and concerns about service capacity. These challenges affect both patients waiting for assessment and treatment, and the NHS services responsible for supporting them. The introduction of NHS England's Neurodevelopmental Data Hub provides a valuable opportunity to better understand patterns of demand using national ADHD data.

This project explored publicly available NHS England ADHD data between December 2024 and December 2025. The aim was not to measure unmet need directly, as the available data does not allow this, but to investigate whether trends in prevalence, referrals and waiting-list activity could provide useful insight into where service pressure may be greatest.

The data was cleaned, prepared and analysed using Python. Trends were examined over time, across age groups and through a simple indicator comparing waiting-list activity with recorded ADHD prevalence.

Several clear patterns emerged from the analysis. Recorded ADHD prevalence increased from approximately 366,000 people to more than 562,000 people during the reporting period, while waiting-list activity increased from approximately 258,000 people to more than 398,000 people. Open referrals also increased by more than 50%.

Adult age groups, particularly those aged 25 and over, accounted for most of this growth. One of the most consistent findings was that adults aged 18–24 and 25+ recorded substantially higher levels of waiting-list demand relative to recorded prevalence than younger age groups. These differences remained remarkably stable throughout the year, suggesting that adult ADHD services may be experiencing greater relative service pressure than children's services.

Although this project cannot determine how many people may require assessment but have not yet entered ADHD pathways, it does provide a useful indication of where service pressure appears to be concentrated.

Based on the findings, four recommendations are proposed:

1. Continue improving ADHD data quality and reporting coverage.
2. Undertake further investigation into adult ADHD service demand.
3. Develop and test more robust measures of unmet demand.
4. Explore demographic differences in ADHD service demand and access.

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## 1. Introduction

I chose this topic because of my interest in healthcare commissioning and service improvement. My interest in this topic is informed both by my professional experience working within the NHS for over ten years, including NHS commissioning roles, and by my personal experience of receiving an ADHD diagnosis as an adult in the UK.

This has given me an additional interest in understanding how demand, waiting lists and access to services are changing over time.

This project uses publicly available NHS England ADHD data to examine trends in prevalence, referrals and waiting-list activity between December 2024 and December 2025.

The aim of the project is not to measure unmet need directly, as the available data does not allow this. Instead, the project explores whether patterns in prevalence, referrals and waiting-list activity can provide useful insight into where service pressure may be greatest and where potential unmet demand may exist.

To answer this question, the data was cleaned, analysed and visualised using Python, with a particular focus on differences between age groups and the relationship between prevalence and waiting-list activity.

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## 2. Background

ADHD services in England have received increasing attention in recent years due to growing demand for assessment and treatment. Alongside increased public awareness, NHS services have reported rising waiting lists and concerns about service capacity, particularly within adult pathways.

The topic has also attracted significant media and public interest, with ongoing discussion around diagnosis rates, access to care and the growing demand for ADHD services. Regardless of these debates, long waiting times can delay access to assessment, treatment and support for individuals seeking help.

In response to these challenges, increasing numbers of patients have accessed assessment through the NHS Right to Choose pathway. While this can improve access to care, it may also create additional financial pressures for NHS commissioners and highlights wider capacity issues within existing ADHD services.

The introduction of NHS England's Neurodevelopmental Data Hub has improved the availability of national ADHD data and provides a new opportunity to explore patterns in prevalence, referrals and waiting-list activity.

This project uses data from the Neurodevelopmental Data Hub to examine how ADHD service demand changed between December 2024 and December 2025 and to explore what these patterns may suggest about potential unmet demand.

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## 3. Methodology

This project uses publicly available ADHD data published by NHS England through the Neurodevelopmental Data Hub.

The data was downloaded, cleaned and analysed using Python.

The analysis focused on four key indicators:

- ADHD prevalence
- Waiting-list activity
- Open referrals
- New referrals

The analysis was carried out in three stages.

First, trends in prevalence, waiting-list activity and referrals were examined over time to understand how ADHD service demand changed during the reporting period.

Second, the data was analysed by age group to explore whether patterns differed across different parts of the population.

Finally, a simple indicator was created by comparing waiting-list activity with recorded ADHD prevalence. This was used as a proxy measure to explore where service pressure may be greatest and where potential unmet demand may exist.

The analysis was primarily descriptive and focused on identifying patterns and trends within the data.

The waiting-list-to-prevalence ratio does not measure unmet need directly. However, it provides a consistent way of comparing demand relative to the known ADHD population across different age groups.

The findings should therefore be interpreted as indicators of relative service pressure rather than absolute levels of unmet demand.

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## 4. Data Sources

The main dataset used in this project was the NHS England ADHD dataset published through the Neurodevelopmental Data Hub.

The dataset contains monthly information on ADHD prevalence, waiting-list activity and referrals across England. Data is reported by age group and reporting period.

I also reviewed the NHS England data dictionary and supporting documentation to better understand how the indicators were defined and collected.

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## 5. Data Preparation and Cleaning

Several data preparation steps were completed before the analysis began.

Column names were simplified, unnecessary fields were removed and date fields were converted into a consistent monthly format. The dataset was then reshaped to support time-series analysis and age-group comparisons.

Records containing suppressed values (for example, values reported as "*") were treated as missing and excluded from calculations where they could affect the results. Basic data quality checks were carried out throughout the process, including reviews of missing values, duplicate records and unusual changes between reporting periods.

Monthly totals were calculated to support trend analysis, while age-group datasets were created to allow comparisons between different population groups. Additional indicators, including the waiting-list-to-prevalence ratio, were developed to explore patterns that may be associated with service pressure.

Particular attention was given to reporting periods where changes in data coverage may have influenced the results. This included review of NHS England documentation relating to the ADHD Data Improvement Plan and known changes in reporting coverage during 2025.

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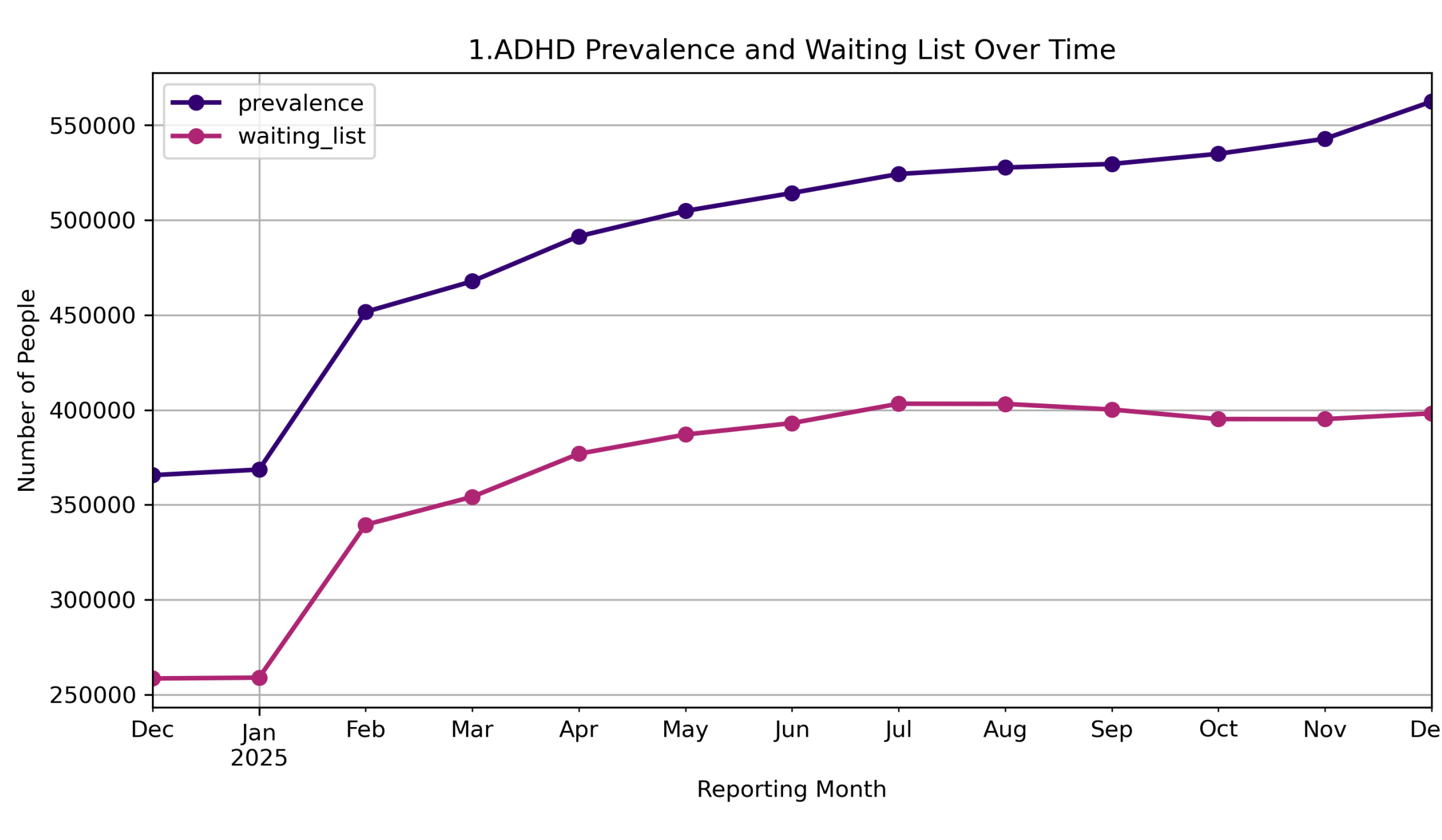
## 6. Exploratory Data Analysis

The exploratory analysis was completed in several stages. I started by looking at overall trends in prevalence, waiting-list activity and referrals before exploring differences between age groups. Finally, I developed a simple indicator to explore patterns that may be associated with potential unmet demand.

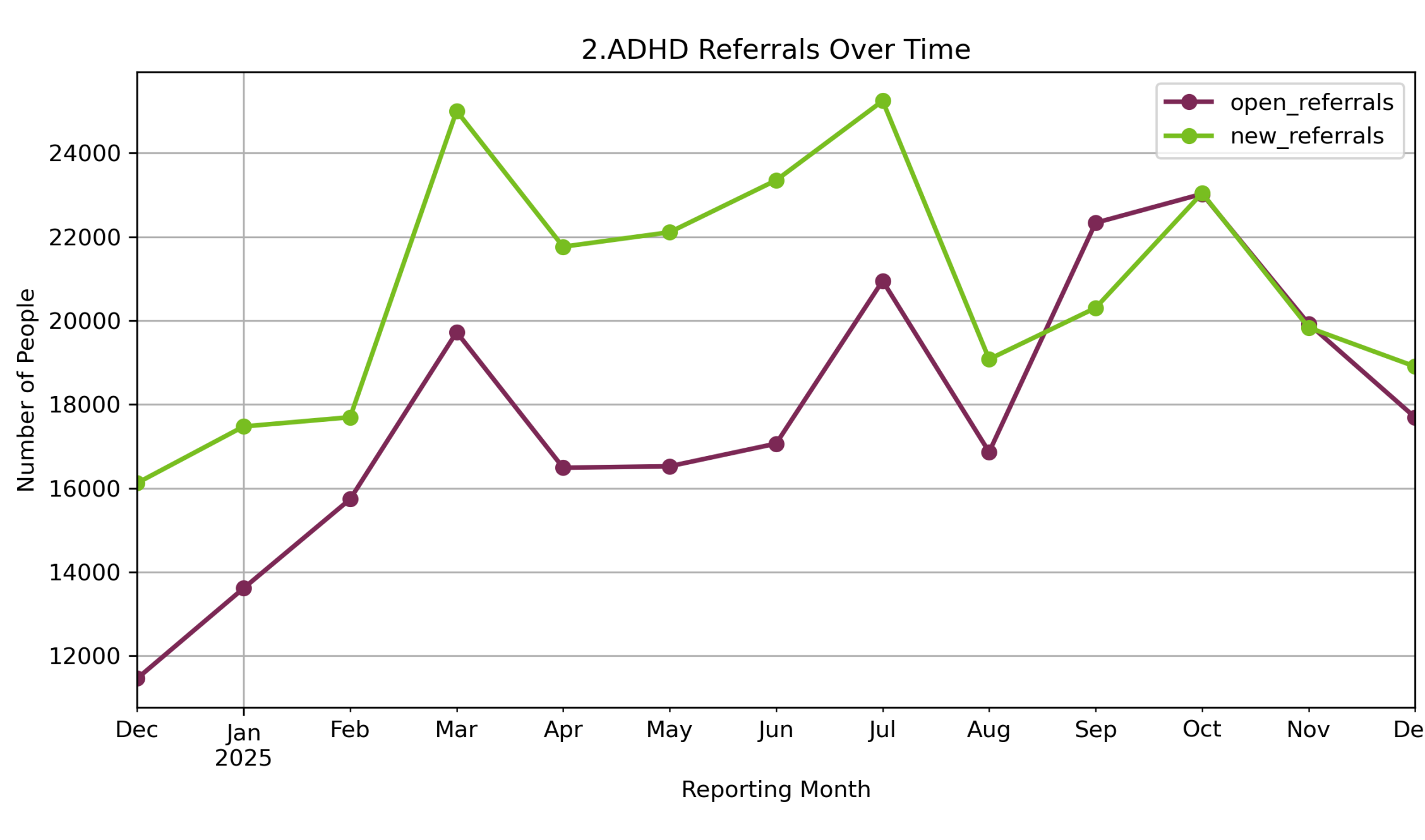
### 6.1 ADHD Prevalence and Waiting Lists Over Time



The first chart compares ADHD prevalence and waiting-list activity across the reporting period. Both measures increased substantially between December 2024 and December 2025.

One of the most noticeable changes occurred between January and February 2025. Based on NHS England documentation, this may partly reflect improvements in reporting coverage associated with the ADHD Data Improvement Plan. The chart suggests that increases in ADHD prevalence were accompanied by similar increases in waiting-list activity.

### 6.2 ADHD Referrals Over Time

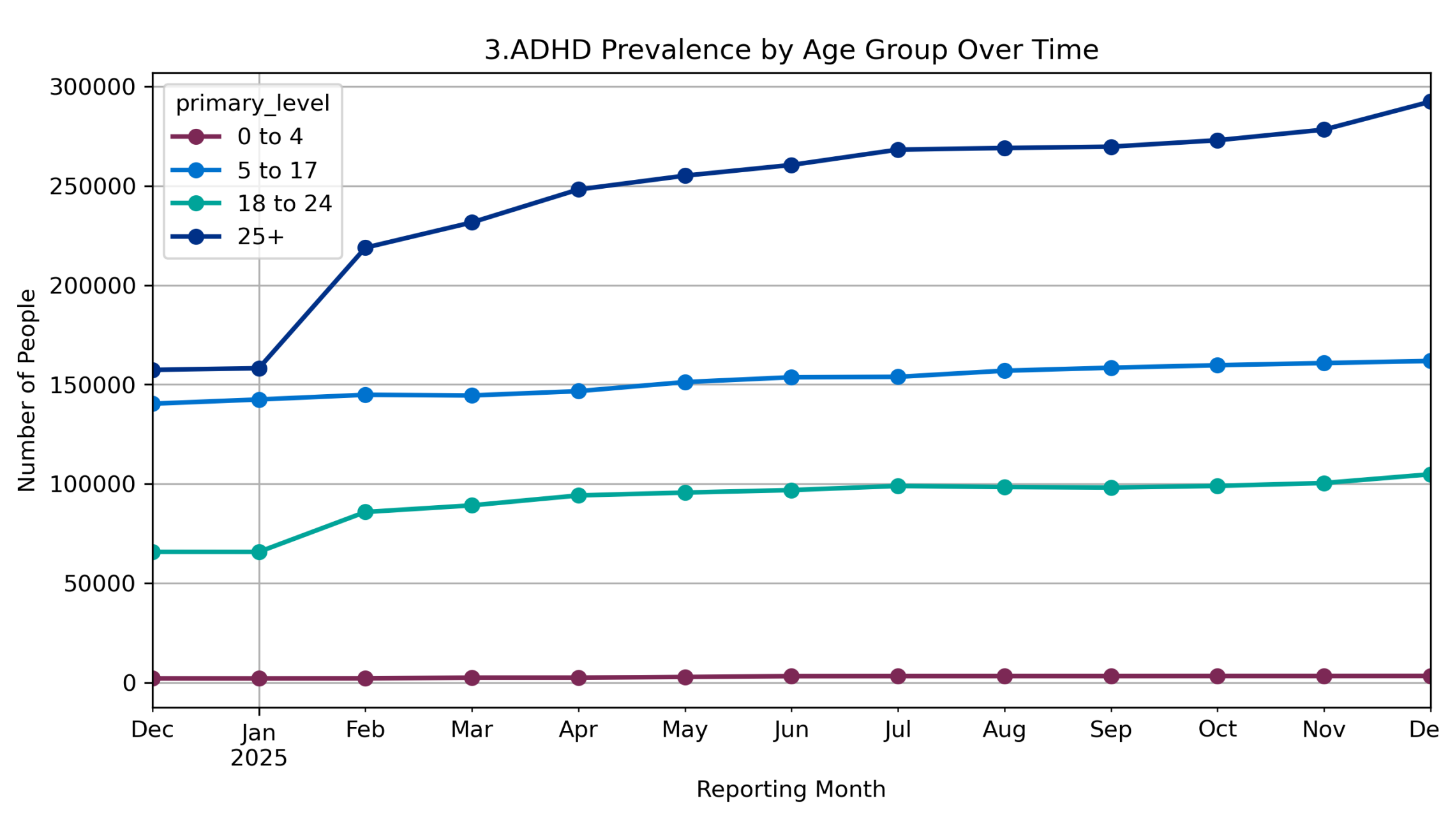


The second chart explores trends in open referrals and new referrals. Open referrals increased steadily throughout the reporting period, while new referrals showed a more gradual increase.

This suggests that demand for ADHD services remained high throughout the year, with services continuing to manage a large number of people already within the referral pathway.

The increase in open referrals alongside continued growth in new referrals may suggest that demand was entering the system faster than services were able to fully process it. However, additional operational data would be required to confirm this interpretation.

### 6.3 ADHD Prevalence by Age Group



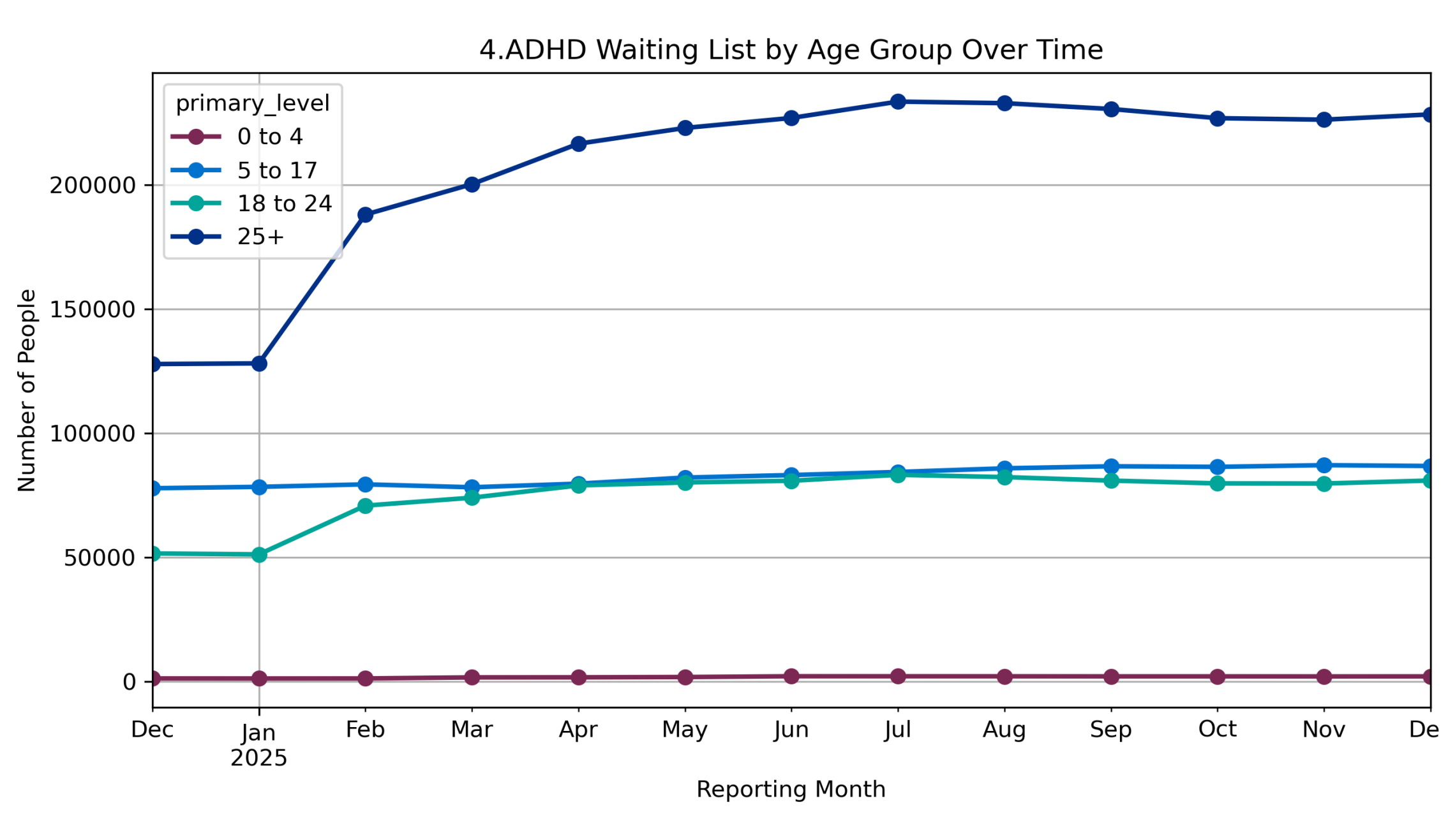
The age-group analysis showed that adults aged 25+ consistently represented the largest proportion of recorded ADHD prevalence throughout the reporting period.

Prevalence within this group increased from approximately 157,000 people in December 2024 to more than 290,000 by December 2025.

The most notable increase occurred between January and February 2025, after which prevalence continued to rise more gradually across the remainder of the year. This pattern mirrors the increase observed in the overall prevalence analysis and may partly reflect improvements in reporting coverage alongside increasing identification of ADHD.

Although prevalence increased across all age groups, the largest absolute growth was observed within the adult age groups, particularly those aged 25 and over. This suggests that adults were the main contributors to the overall increase in recorded ADHD prevalence during the reporting period.

### 6.4 ADHD Waiting Lists by Age Group

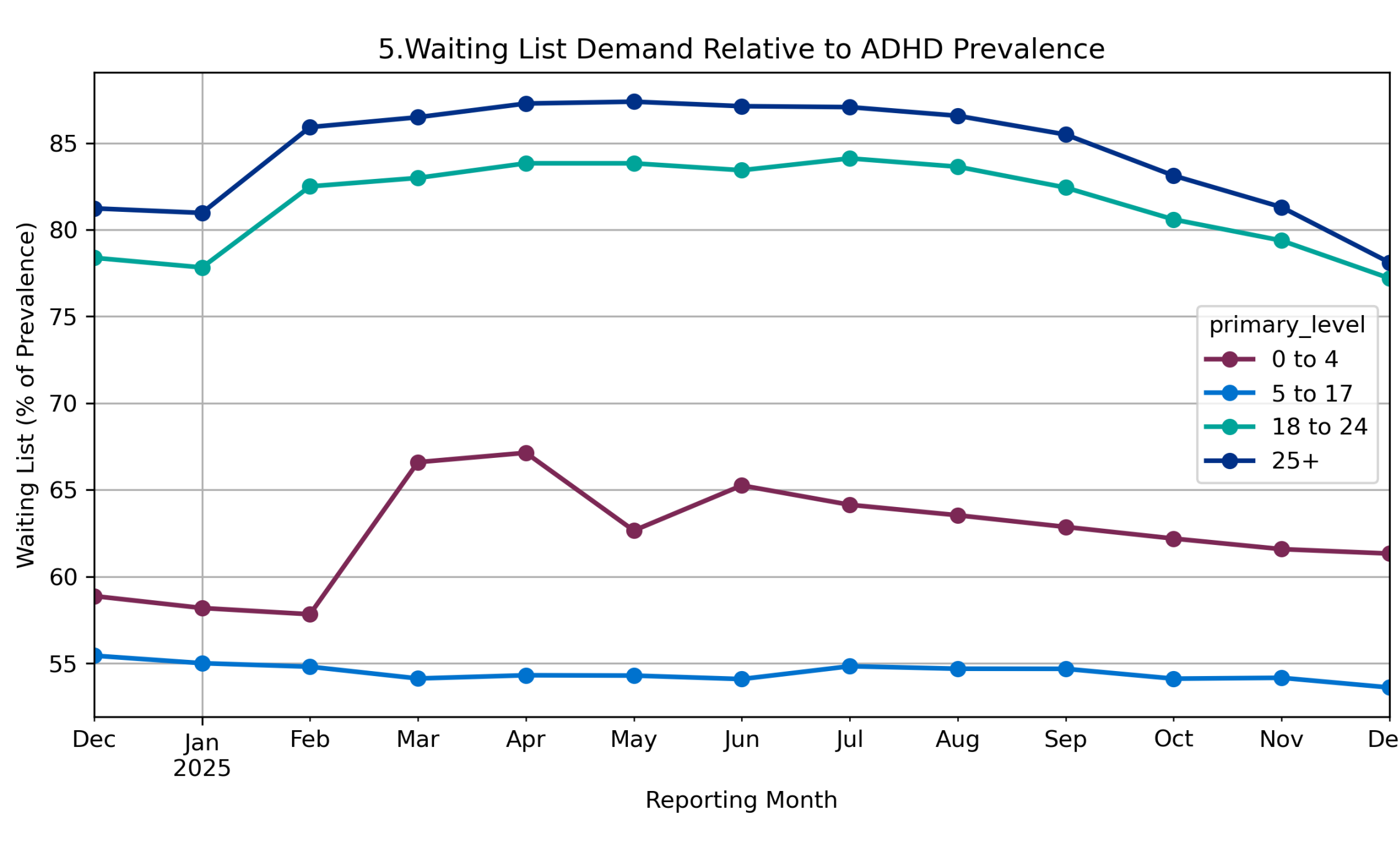


A similar pattern was observed when examining waiting-list activity by age group. Adults aged 25+ consistently recorded the largest waiting lists, increasing from approximately 128,000 people in December 2024 to more than 225,000 during 2025.

Waiting lists also increased substantially within the 18–24 age group, while the 5–17 age group experienced more modest growth. In contrast, the 0–4 age group remained very small throughout the reporting period.

These findings suggest that much of the increase in waiting-list demand was concentrated within adult ADHD services. This aligns with the prevalence analysis and indicates that rising demand may be affecting adult pathways more significantly than children's services.

### 6.5 Waiting-List Demand Relative to ADHD Prevalence

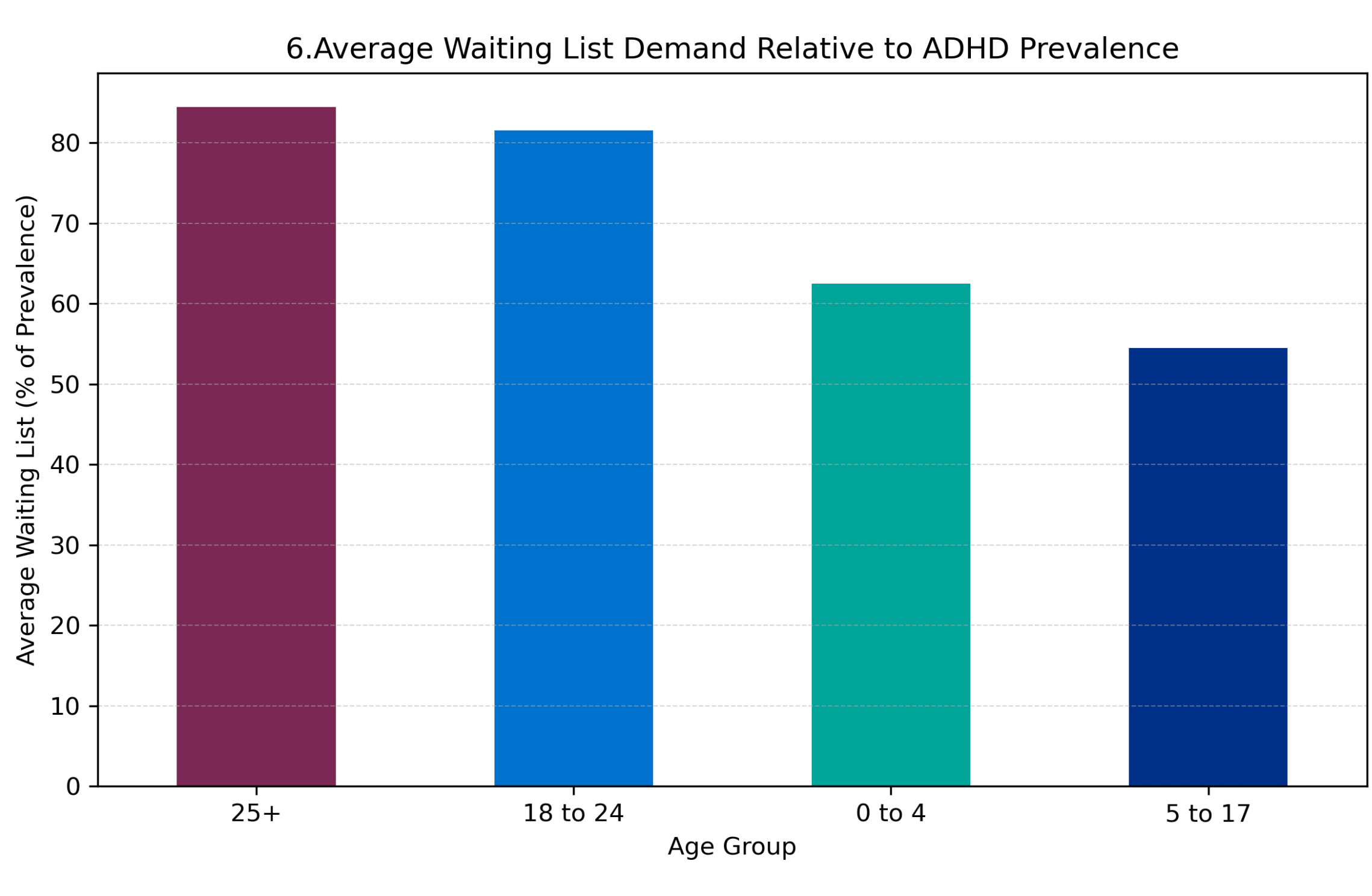


An additional observation is that the gap between adult and child services was not only large but also remarkably stable throughout the year.

Adult age groups consistently recorded substantially higher waiting-list-to-prevalence ratios than younger age groups, with relatively little variation between reporting periods.

This suggests that the pattern is unlikely to be explained by a single reporting month, data anomaly or short-term fluctuation. Instead, it appears to represent a more sustained difference in service demand and service pressure between adult and children's ADHD pathways.

### 6.6 Average Waiting-List Demand Relative to ADHD Prevalence

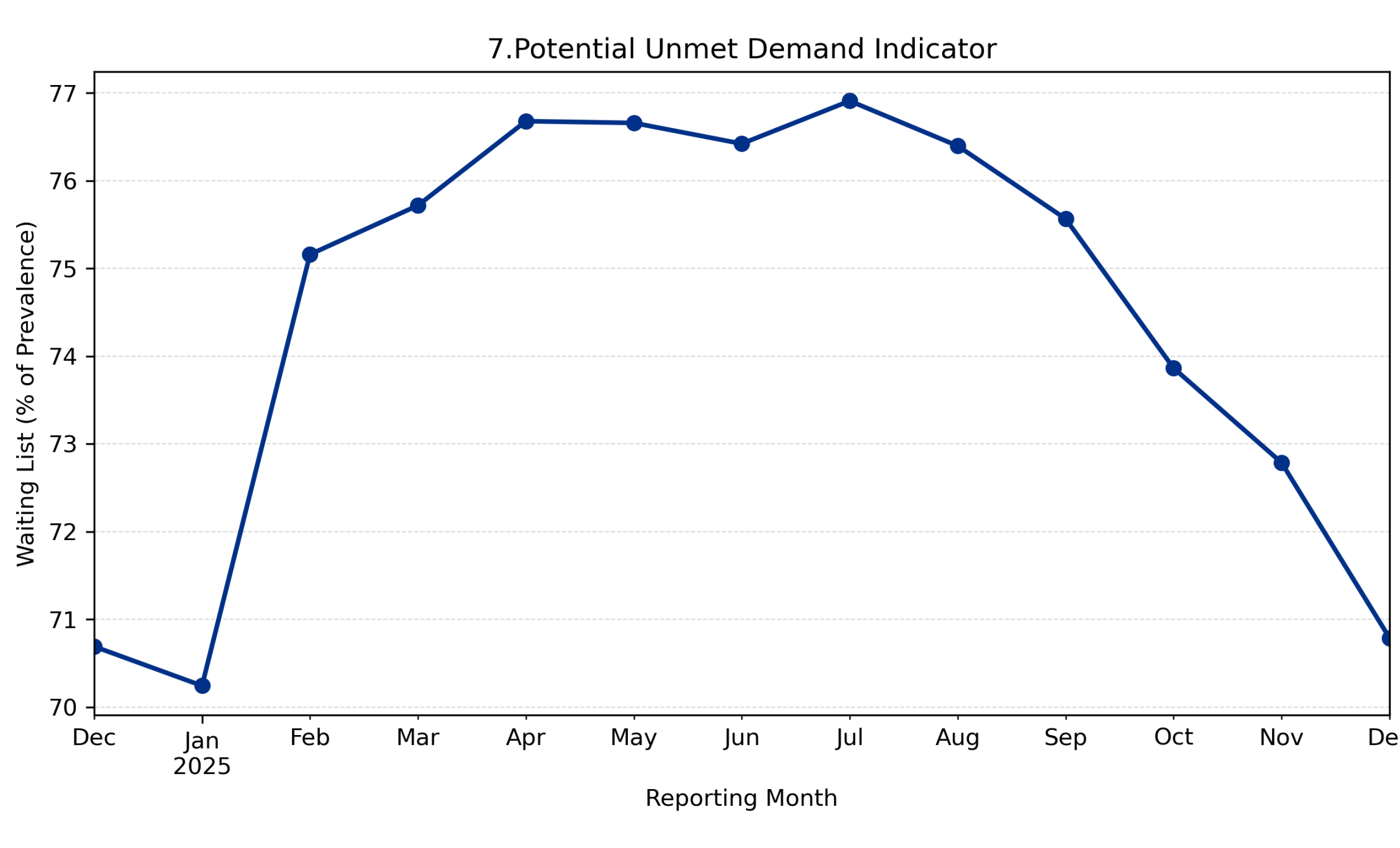


The average ratio for each age group was calculated to provide a simpler comparison.

Adults aged 25+ had the highest average ratio, followed closely by the 18–24 age group. Younger age groups showed substantially lower ratios.

This reinforced the earlier finding that adult ADHD services may be experiencing greater relative service pressure. The relatively small differences observed between the adult age groups suggest that increased service pressure may not be limited to a single adult cohort but may instead affect adult ADHD pathways more broadly.

### 6.7 Potential Unmet Demand Indicator



Finally, a simple indicator was created by comparing waiting-list activity with recorded ADHD prevalence across all age groups.

The ratio remained consistently high throughout the reporting period, suggesting that waiting-list activity represented a substantial proportion of recorded prevalence.

While this should not be interpreted as a direct measure of unmet need, it provides a useful indication of service pressure and highlights the relationship between prevalence and waiting-list demand.

### 6.8 EDA Summary

The exploratory analysis identified several consistent patterns across the dataset. ADHD prevalence, waiting-list activity and referral volumes all increased during the reporting period between December 2024 and December 2025.

Three findings stood out in particular. First, both recorded ADHD prevalence and waiting-list activity increased substantially across the year. Second, adult age groups, especially those aged 25 and over, accounted for most of this growth. Third, adult services consistently showed higher waiting-list demand relative to recorded prevalence than children's services.

This pattern was observed across multiple charts and remained consistent regardless of whether prevalence, waiting-list activity or waiting-list ratios were examined.

The difference between adult and children's services remained remarkably stable throughout the reporting period, suggesting that the pattern was unlikely to be driven by a single reporting month or short-term fluctuation.

Taken together, these findings suggest that service pressure may be concentrated within adult ADHD pathways and provide a useful starting point for exploring potential unmet demand. These observations informed the more detailed findings presented in the following section.

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## 7. Key Findings

The analysis identified several important patterns across the ADHD dataset between December 2024 and December 2025.

| Indicator | Dec 2024 | Dec 2025 | Change |
|---|---|---|---|
| ADHD prevalence | 366,000 | 562,000 | +54% |
| Waiting-list activity | 258,000 | 398,000 | +54% |
| Open referrals | 11,000 | 18,000 | +54% |

### 7.1 Growth in ADHD Prevalence

Recorded ADHD prevalence increased substantially during the reporting period, rising from approximately 366,000 people in December 2024 to more than 562,000 people by December 2025, representing an increase of around 54%.

While part of this increase may reflect improvements in reporting coverage associated with NHS England's ADHD Data Improvement Plan, the scale and consistency of the growth suggest that increasing numbers of people are being identified and supported by ADHD services across England.

The increase was particularly noticeable between January and February 2025, after which prevalence continued to rise throughout the remainder of the year.

### 7.2 Growth in Waiting-List Activity and Referrals

Waiting-list activity also increased considerably during the same period, growing from approximately 258,000 people to more than 398,000 people, an increase of around 54%.

Open referrals increased by more than 50%, while new referrals showed a smaller but still notable increase.

Taken together, these findings suggest that demand for ADHD assessment and treatment remained high throughout the year. Services were not only receiving new referrals but were also continuing to manage large numbers of people already waiting within the pathway.

The similarity between the growth observed in prevalence, waiting-list activity and open referrals is notable and suggests that increasing identification of ADHD was accompanied by increasing demand for assessment and treatment services.

### 7.3 Adult Age Groups Showed the Highest Levels of Service Pressure

One of the clearest findings was the difference between age groups.

The largest prevalence figures and the largest waiting lists were consistently observed within the adult age groups, particularly among people aged 25 and over.

Adults aged 25+ accounted for most of the observed growth in both prevalence and waiting-list activity during the reporting period. The 18–24 age group also experienced substantial increases, while younger age groups showed more modest growth.

When waiting-list activity was compared with recorded prevalence, adults aged 18–24 and 25+ consistently showed substantially higher ratios than younger age groups.

This suggests that adult ADHD services may be experiencing greater relative service pressure than children's services, which could indicate a mismatch between growing demand and available service capacity within adult pathways.

### 7.4 The Patterns Remained Remarkably Stable Over Time

Another interesting finding was how consistent these differences remained throughout the reporting period.

Although prevalence, referrals and waiting-list activity changed over time, the relationship between waiting-list demand and prevalence remained broadly unchanged. The adult age groups consistently showed the highest ratios, while the younger age groups consistently showed the lowest.

This suggests that the findings are unlikely to be explained by a single reporting month or short-term fluctuation and may instead reflect more established patterns within ADHD services.

The stability of these findings across multiple analyses strengthens confidence that the observed differences represent a sustained pattern rather than a temporary change in reporting or operational activity.

### 7.5 Potential Unmet Demand Appears Greatest Within Adult Services

The project was not able to measure unmet need directly. However, the waiting-list-to-prevalence indicator provided a useful way of exploring where service pressure may be greatest.

Across multiple analyses, the adult age groups consistently showed higher levels of waiting-list demand relative to recorded prevalence.

While further investigation would be needed to understand the reasons behind this pattern, the findings suggest that potential unmet demand may be greatest within adult ADHD services.

The indicator should not be interpreted as a direct measure of unmet need. However, it provides a useful indication of where service pressure appears to be concentrated and highlights areas that may benefit from further investigation.

In particular, the findings suggest that adult ADHD pathways may warrant closer examination when considering future service planning and resource allocation.

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## 8. Limitations

This project uses aggregate national data and does not include individual patient records. As a result, it was not possible to explore patient-level factors or understand the experiences of individual people within ADHD pathways.

The analysis is also based on indicators published by NHS England and therefore depends on the quality and completeness of the source data. Changes in reporting coverage may mean that some of the increases observed during the reporting period reflect improved data submission as well as genuine increases in activity.

This is particularly relevant when interpreting the notable increase observed between January and February 2025.

To explore potential unmet demand, I used the relationship between waiting-list activity and recorded ADHD prevalence as a proxy measure. While this provided useful insights into relative service pressure, it should not be interpreted as a direct measure of unmet need.

The waiting-list-to-prevalence ratio may reflect a combination of unmet demand, service backlog, referral behaviour, diagnostic capacity, service capacity constraints and reporting practices.

It should therefore be interpreted as a broad indicator of service pressure rather than a direct measure of unmet need. While the ratio provides a useful way of comparing patterns across age groups, it cannot determine the underlying causes of the observed differences.

The project also covers a relatively short period of time, spanning just over one year. As a result, longer-term trends could not be explored and some changes may appear more significant than they would over a longer reporting period.

Finally, the analysis focused primarily on trends over time and differences between age groups. Other potentially important factors, such as ethnicity, sex, deprivation and geographical variation, were outside the scope of this project and may provide useful areas for future research.

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## 9. Conclusions

This project explored whether routinely published NHS ADHD data could be used to provide insight into potential unmet demand for ADHD assessment services in England.

Using data from the NHS Neurodevelopmental Data Hub, the analysis examined trends in recorded ADHD prevalence, waiting-list activity and referral patterns between December 2024 and December 2025.

The findings showed substantial growth in both recorded prevalence and waiting-list activity during the reporting period, alongside continued increases in referrals.

A key finding was the consistent difference observed between age groups. Adult services, particularly those supporting people aged 18–24 and 25+, showed higher levels of waiting-list demand relative to recorded prevalence than younger age groups.

These patterns remained remarkably stable throughout the year, suggesting they are unlikely to be explained by a single reporting period or short-term fluctuation.

The project was not able to measure unmet need directly and therefore cannot conclude how many people may require assessment but have not yet entered ADHD pathways.

However, the analysis demonstrates that comparing waiting-list activity with recorded prevalence can provide a useful indication of where service pressure appears to be greatest.

Taken together, the findings suggest that adult ADHD services may be experiencing disproportionately higher levels of demand relative to the size of the known ADHD population.

This highlights the importance of continued investment in ADHD data quality, pathway development and service planning.

More broadly, the project demonstrates how routinely collected NHS data can be used to generate practical insights for commissioners, providers and policy-makers.

While further work would be needed to develop a more robust measure of unmet demand, the approach used in this project provides a useful starting point.

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## 10. Recommendations

### Recommendation 1: Continue Improving ADHD Data Quality and Reporting Coverage

The analysis identified a substantial increase in recorded ADHD prevalence and waiting-list activity between January and February 2025.

This may partly reflect improvements in data coverage and reporting following NHS England's ADHD Data Improvement Plan.

Continued work to improve the completeness, consistency and standardisation of ADHD data would support more accurate monitoring of demand and help ensure future analyses are based on increasingly robust information.

### Recommendation 2: Undertake Further Investigation into Adult ADHD Service Demand

Adult age groups consistently showed higher waiting-list demand relative to recorded ADHD prevalence than children's services.

This pattern remained stable throughout the reporting period and was particularly noticeable within the 18–24 and 25+ age groups.

Further investigation should be undertaken to better understand the factors contributing to this higher level of relative demand.

This could include targeted analysis of adult ADHD pathways, including referral-to-treatment times, provider capacity, diagnostic throughput and Right to Choose activity.

A better understanding of these factors may help identify where service pressures are greatest and inform future pathway development and resource planning.

### Recommendation 3: Develop and Test More Robust Measures of Unmet Demand

This project used the ratio between waiting-list activity and recorded ADHD prevalence as a simple proxy indicator of potential unmet demand.

While this approach provides useful insight into service pressure, it cannot directly measure unmet need.

Future work could explore the development of more sophisticated indicators by combining ADHD prevalence data with population estimates, referral activity, service capacity information and diagnostic throughput data.

This may help provide a more accurate understanding of unmet demand and support future service planning.

### Recommendation 4: Explore Demographic Differences in ADHD Service Demand and Access

This project focused primarily on trends over time and differences between age groups.

Future work could explore demographic differences in ADHD prevalence, waiting-list activity and service access, including analysis by ethnicity, sex, deprivation and geography.

This may help identify potential inequalities in access to assessment and treatment, improve understanding of unmet need across different population groups and support more targeted service improvement.

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